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Executive Summary

This survey of 280 oncologists across Germany, France, Italy, Spain, United Kingdom examined how clinicians perceive Keytrava (pemvrolimab) for non-small cell lung cancer. Fieldwork took place in May 2023.

Overall HCP sentiment index for Keytrava in France: 0.37 (scale 0 to 1, assessed 2023). Respondents viewed the brand negatively.

Methodology

Respondents were oncologists who treat at least 17 patients per month with non-small cell lung cancer. The sample size was n=280. Interviews lasted 27 minutes and were conducted online.

Results are weighted to national prescribing volumes. Statistical significance was assessed at the 95% confidence level.

The unmet need in non-small cell lung cancer persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Registry data show that outcomes in non-small cell lung cancer depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Peer-to-peer education and congress data were cited as the most trusted sources when oncologists evaluate new evidence on PD-1 inhibitor agents.

Oncologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between PD-1 inhibitor options.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Key Findings

Adherence concerns centred on discontinuation after early adverse events, particularly in patients with comorbidities and polypharmacy.

Oncologists in France rated Keytrava tolerability profile as negatively, citing immune-related adverse events and infusion reactions as the main concerns among patients with non-small cell lung cancer.

62% of respondents considered the efficacy of Keytrava clinically meaningful versus the current standard of care, with the strongest support in second-line use.

Many oncologists said they would like more information about Keytrava.

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Dose modification guidance for Keytrava is generally followed, but oncologists said that practical monitoring of immune-related adverse events depends on local capacity and patient travel distance.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

Supply and logistics are not a concern for Keytrava, though cold-chain handling and nurse-led administration add workload in busy clinics.

Guideline committees are expected to review the evidence base for non-small cell lung cancer again next year, which could change eligibility criteria and referral patterns.

Conclusions

Keytrava is perceived negatively in France. Priorities for the brand team are discontinuation after early adverse events and evidence generation for non-small cell lung cancer.

Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Safety monitoring focused on immune-related adverse events and infusion reactions, with dose interruption or discontinuation used for severe adverse events.

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Cost-effectiveness submissions typically model first-line use, and payers question the durability of benefit beyond the trial follow-up period.

Synthetic document generated for the Watson prototype. All brand names and figures are fictional.